

D407

Models of Care and Healthcare Trends

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A regional health system that owns two hospitals buys a home health agency and a rehab center. Its CEO has led the system for a decade and it just rebranded. What kind of integration is this?

- A. A PCMH
- B. Horizontal integration
- C. Vertical integration
- D. Fragmentation

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

A family practice with 8,000 patients adds a care coordinator, same-day appointments, and a team of a physician, nurse, and behavioral health specialist. It also repainted its offices. What model is it adopting?

- A. Home health care
- B. Hospital-at-home
- C. A patient-centered medical home
- D. An HMO

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

An ACO's spending benchmark for its population is \$50 million. Actual spending is \$46 million, and it met all quality standards. Its sharing rate is 50%. The ACO includes 12 clinics and one hospital. How much does it earn in shared savings?

- A. \$2 million
- B. \$23 million
- C. \$0, because the ACO must save at least 50%.
- D. \$4 million

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

An ACO with a \$30 million benchmark and a 40% sharing rate earned \$1.2 million in shared savings after meeting quality. What was actual spending?

- A.** \$33 million
- B.** \$3 million
- C.** \$27 million
- D.** \$28.8 million

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A woman with an HMO plan wants to see a dermatologist without first seeing her primary care doctor. The dermatologist is out of network. She has had the plan for 5 years and loves its app. What is most likely?

- A.** Covered with no referral because specialists are always direct access.
- B.** She needs a referral, and the out-of-network visit generally will not be covered.
- C.** Covered at a higher cost share.
- D.** Covered in full because she has been a member for years.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

A patient wants to see any specialist without a referral and keep some coverage out of network. Which plan fits?

- A.** EPO
- B.** POS
- C.** HMO
- D.** PPO

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A dermatologist in a large city reviews photos of a rural patient's rash sent by the clinic yesterday and replies with a treatment plan today. The clinic has fast internet. What telehealth modality is this?

- A. Remote patient monitoring
- B. Synchronous video visit
- C. An in-person consult
- D. Asynchronous (store-and-forward)

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8. [TRANSFER — reversed stem / different variable]

A heart failure patient uses a home scale that sends daily weights to a nurse who calls if weight jumps. What modality is this?

- A. Remote patient monitoring
- B. Retail clinic care
- C. Synchronous telehealth
- D. Store-and-forward

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A healthy 45-year-old needs a routine cataract surgery. Her surgeon operates at both a large hospital and an ambulatory surgery center; the hospital has a new parking garage. Which setting fits current trends?

- A. The hospital, because hospitals are always safest.
- B. The ambulatory surgery center.
- C. An inpatient admission for observation.
- D. The hospital, for the new garage.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A health system launches a program to reduce clinician burnout by cutting documentation time. The system recently opened a cafeteria. Which aim does this address?

- A. The fourth aim in the Quadruple Aim: clinician well-being.
- B. Population health.
- C. None; burnout is outside system goals.
- D. Lower per capita cost only.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11. [TRANSFER — reversed stem / different variable]

Which element distinguishes the Quintuple Aim from the Quadruple Aim?

- A. Clinician well-being
- B. Health equity
- C. Patient experience
- D. Lower cost

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A health plan finds 5% of members account for about half of spending. The plan has 200,000 members and a new mobile app. How should care management resources be focused?

- A. Spread evenly across all members.
- B. Through the new app for everyone.
- C. Only on healthy members to keep them healthy.
- D. On the highest-risk members, through risk stratification.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer C Concept 1

A regional health system that owns two hospitals buys a home health agency and a rehab center. ~~Its CEO has led the system for a decade and it just rebranded.~~ What kind of integration is this?

Target word: integration type **Decides:** Different levels of the continuum.

A	Adjacent category	A primary care model, not a merger type.
B	Confusion set	Would be buying more hospitals.
C	Correct	Joins different care levels.
D	Inverted operation	The opposite of integration.

2. Answer C Concept 2

A family practice ~~with 8,000 patients~~ adds a care coordinator, same-day appointments, and a team of a physician, nurse, and behavioral health specialist. ~~It also repainted its offices.~~ What model is it adopting?

Target word: model name **Decides:** Team-based coordinated primary care.

A	Confusion set	Services in the patient's home.
B	Adjacent category	Acute care in the home.
C	Correct	Team-based, accessible, coordinated primary care.
D	Adjacent category	An insurance plan type.

3. Answer A Concept 3

An ACO's spending benchmark for its population is \$50 million. Actual spending is \$46 million, and it met all quality standards. Its sharing rate is 50%. ~~The ACO includes 12 clinics and one hospital.~~ How much does it earn in shared savings?

Target word: compute shared savings **Decides:** Savings $\$4\text{M} \times 50\%$.

A	Correct	$(\$50\text{M} - \$46\text{M}) \times 50\% = \$2\text{M}.$
B	Wrong stem number	50% of actual spending.
C	Absolute language	Misreads the sharing rate as a savings requirement.
D	Dropped element	Leaves out the sharing rate.

4. Answer C Concept 3 • transfer

An ACO with a \$30 million benchmark and a 40% sharing rate earned \$1.2 million in shared savings after meeting quality. What was actual spending?

Target word: solve for actual spending **Decides:** Savings = $\$1.2\text{M} / 0.4 = \$3\text{M}.$

A	Inverted operation	Adds savings to the benchmark.
B	Wrong stem number	The savings, not spending.
C	Correct	$\$30\text{M} - \$3\text{M} = \$27\text{M}.$
D	Dropped element	Subtracts the payment instead of total savings.

5. Answer B Concept 4

A woman with an HMO plan wants to see a dermatologist without first seeing her primary care doctor. The dermatologist is out of network. ~~She has had the plan for 5 years and loves its app.~~ What is most likely?

Target word: HMO rules **Decides:** Gatekeeper and network-only.

A	Absolute language	HMOs require referrals.
B	Correct	HMO rules.
C	Confusion set	That is a PPO.
D	Camouflage	Tenure doesn't change rules.

6. Answer D Concept 4 • transfer

A patient wants to see any specialist without a referral and keep some coverage out of network. Which plan fits?

Target word: reverse: from features to plan **Decides:** No referral plus out-of-network coverage.

A	Confusion set	No referrals but network-only.
B	Confusion set	Usually requires referrals.
C	Inverted operation	Requires referrals and is network-only.
D	Correct	Allows both.

7. Answer D Concept 5

A dermatologist in a large city reviews photos of a rural patient's rash sent by the clinic yesterday and replies with a treatment plan today. ~~The clinic has fast internet.~~ What telehealth modality is this?

Target word: classify modality **Decides:** Not real-time.

A	Confusion set	Continuous readings from home devices.
B	Inverted operation	Requires real-time interaction.
C	Adjacent category	No physical visit.
D	Correct	Data sent and reviewed later.

8. Answer A Concept 5 • transfer

A heart failure patient uses a home scale that sends daily weights to a nurse who calls if weight jumps. What modality is this?

Target word: reverse: classify **Decides:** Data from home devices.

A	Correct	Home device readings transmitted.
B	Adjacent category	A site of care.
C	Inverted operation	Live visits.
D	Confusion set	Images or records sent for review.

9. Answer B Concept 6

A healthy 45-year-old needs a routine cataract surgery. Her surgeon operates at both a large hospital and an ambulatory surgery center; ~~the hospital has a new parking garage.~~ Which setting fits current trends?

Target word: site of care **Decides:** Routine procedure, healthy patient.

A	Absolute language	Absolute; ASCs are safe for routine cases.
B	Correct	Lower cost and convenient when safe.
C	Inverted operation	Moves care up, not down.
D	Camouflage	Parking doesn't decide the setting.

10. Answer A Concept 7

A health system launches a program to reduce clinician burnout by cutting documentation time. ~~The system recently opened a cafeteria.~~ Which aim does this address?

Target word: map goal to aim **Decides:** Clinician well-being.

A	Correct	Burnout reduction.
B	Confusion set	Another aim.
C	Absolute language	The Quadruple Aim includes it.
D	Dropped element	Leaves out the well-being focus.

11. Answer B Concept 7 · transfer

Which element distinguishes the Quintuple Aim from the Quadruple Aim?

Target word: reverse; identify the added aim **Decides:** Health equity.

A	Inverted operation	The fourth aim.
B	Correct	The fifth aim.
C	Confusion set	Part of the Triple Aim.
D	Confusion set	Part of the Triple Aim.

12. Answer D Concept 8

A health plan finds 5% of members account for about half of spending. ~~The plan has 200,000 members and a new mobile app.~~ How should care management resources be focused?

Target word: targeting **Decides:** Concentrated spending.

A	Inverted operation	Ignores concentration.
B	Camouflage	The app doesn't decide targeting.
C	Adjacent category	Wellness matters, but the largest gains come from the highest-risk group.
D	Correct	Targeting yields greatest impact.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	3	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	6	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	8	
Absolute language	Always, never, guarantee — where judgment applies.	4	
Confusion set	Every option is real; the test is exact role-matching.	10	
Camouflage	Answers a stem fact that was never the question.	3	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Vertical vs. horizontal integration	Vertical integration joins different levels of the care continuum (clinics, hospital, rehab, home health) under one system; horizontal integration joins organizations at the same level (hospital with hospital).	Calling a merger of two hospitals vertical integration.	A merger or acquisition described by the types of organizations joined.
2	Patient-centered medical home	A PCMH is team-based primary care that is comprehensive, coordinated across settings, accessible (extended hours, same-day slots), and focused on quality and safety.	Thinking a medical home is a building or a home health service.	A primary care practice redesign with teams and care coordination.
3	ACO shared savings	An ACO earns a share of savings when actual spending for its attributed population falls below the benchmark and quality standards are met: $\text{payment} = (\text{benchmark} - \text{actual}) \times \text{sharing rate}$.	Thinking the ACO keeps all savings or earns savings when quality targets are missed.	A benchmark, actual spending, a sharing rate, and quality results.
4	Managed care plan types	HMOs use a primary care gatekeeper, require referrals, and cover in-network care only (except emergencies); PPOs allow out-of-network care at higher cost with no referral; EPOs are network-only without referrals; POS plans mix HMO referrals with some out-of-network coverage.	Believing HMOs cover out-of-network care or that PPOs require referrals.	Referral requirements and out-of-network coverage.
5	Telehealth modalities	Synchronous telehealth is real-time video or phone; asynchronous (store-and-forward) sends images or data for later review; remote patient monitoring transmits readings such as blood pressure or glucose from home.	Calling a sent photo reviewed hours later a live visit.	Timing of the exchange and who acts when.
6	Shifting sites of care	Care is moving from inpatient hospitals to lower-cost, convenient settings — ambulatory surgery centers, urgent care, retail clinics, and hospital-at-home — when safety and quality allow.	Assuming the hospital is always the highest-quality setting.	A routine procedure or stable condition and a choice of setting.

#	Concept	Rule	Trap	Tell
7	Triple, Quadruple, and Quintuple Aim	The Triple Aim is better patient experience, better population health, and lower per capita cost; the Quadruple Aim adds clinician well-being; the Quintuple Aim adds health equity.	Leaving out cost or thinking clinician burnout is outside system goals.	A goal to map to an aim.
8	Population health risk stratification	A small share of patients drives a large share of costs, so systems stratify their population by risk and target the highest-risk group for care management.	Spreading care management evenly across all patients.	Spending concentrated in a small group.

Course scope (public description): Models of Care and Healthcare Trends is a course for health professionals in a variety of roles in the health and human services industry, which examines the unique characteristics of healthcare models in the United States and emerging trends created by social and political drivers. The course explores the evolution of healthcare models from fragmented systems to cohesive, quality-centric, and client-focused systems. The course also focuses on innovative trends, such as access to care, telemedicine, and subsequent shifts in the continuum of care as it relates to patient or client outcomes. There are no prerequisites for this course.